

# Spinal Health Assessment Summary

A shared clinical summary for Pending and their care team

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CONFIDENTIAL • PERSONAL HEALTH INFORMATION

## PRIMARY IMPRESSION

### Persistent mechanical low back pain after lifting injury

Ongoing low back pain from the muscles, joints, or discs

Moderate confidence

Lumbar spine

Your pain began after deadlifting with a pop and severe short-term loss of movement, and it has remained mostly in the lower back for about three to six months. The sharp catches with sitting and twisting, improvement with walking or changing position, and absence of reported leg or neurologic symptoms fit a mechanical low-back pain pattern, but the exact pain generator cannot be confirmed without an examination and possibly imaging.

**Why this confidence:** The history supports a mechanical lumbar pain syndrome, but there is no imaging and no examination data identifying the specific tissue involved. The stable course and absence of reported neurologic or systemic red flags make a dangerous cause less likely, while an in-person assessment remains appropriate because symptoms have persisted for several months.

**ALSO CONSIDERED** Lumbar disc-related pain without radiculopathy • Facet or sacroiliac-region mechanical pain

## URGENCY & SAFETY

### ● Routine specialist evaluation

Arrange an in-person primary-care, sports-medicine, or physical-medicine evaluation within 2–4 weeks.

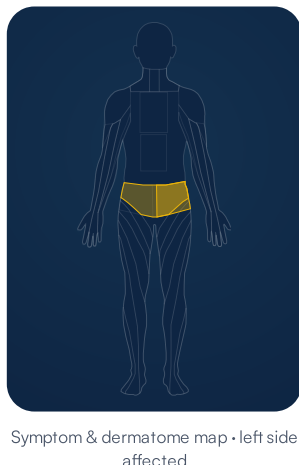
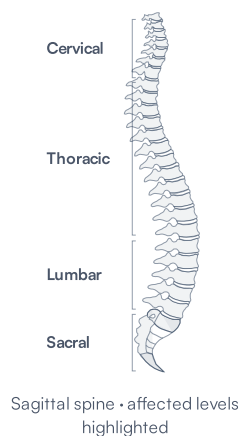
Your pain has persisted for about three to six months after a lifting injury and is affecting sleep and confidence with returning to the gym. You do not report leg symptoms, neurologic symptoms, or systemic warning signs, so this does not appear to require emergency care, but an in-person evaluation within the next few weeks is appropriate to examine your back and decide whether imaging or structured rehabilitation is needed.

## SEEK URGENT CARE IF

- New or worsening leg weakness
- New numbness or pain spreading down a leg
- Loss of bladder or bowel control
- Numbness in the groin or saddle area
- Inability to walk safely
- Fever, unexplained weight loss, or severe unrelenting pain at rest

Contact your doctor urgently for a same-week evaluation. If symptoms rapidly worsen, seek emergency care right away. If you develop new leg weakness, loss of bladder or bowel control, numbness in the groin or saddle area, or inability to walk safely, call 911 or go to the nearest emergency department.

## WHERE IT'S HAPPENING



### AFFECTED LEVELS

No discrete levels identified.

### TRACKED SYMPTOMS

- **Low back pain**  
Axial mechanical low back pain • stable • About three to six months
- **Sharp catching pain with movement**  
Movement-provoked sharp lumbar pain • fluctuating • About three to six months
- **Sleep difficulty from back pain**  
Pain-related sleep-onset difficulty • stable • About three to six months

● Mild ● Moderate ● Severe ● Worst

## WHAT THIS MEANS

Axial mechanical low back pain means pain centered in the lower back rather than pain traveling down a leg from a nerve. It can arise from irritated muscles, spinal joints, ligaments, or discs, and these sources often overlap. The term describes the symptom pattern and does not by itself identify one damaged structure.

### OTHER CONDITIONS CONSIDERED

- **Lumbar disc-related pain without radiculopathy** — A disc-related pain source is possible because symptoms began with lifting and twisting and are aggravated by sitting, but there is no reported leg pain or neurologic pattern to confirm nerve involvement.
- **Facet or sacroiliac-region mechanical pain** — A small spinal-joint or sacroiliac-region pain generator could cause one-sided low back pain and sharp catches with twisting; examination is needed to distinguish it from disc or muscle pain.

## YOUR CARE PATHWAY

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**Goals of care:** The goals are to confirm the pain pattern, restore comfortable movement, reduce flare-ups, and help you return to work and exercise gradually without assuming that surgery is needed.

### Start with an in-person evaluation, then build a graded rehabilitation plan

Because your low back pain has lasted several months after lifting and is affecting sleep and confidence with exercise, an in-person examination within 2–4 weeks is appropriate. There is no current report of leg or neurologic symptoms, so the next step is evaluation and conservative rehabilitation rather than emergency care or surgery.

- **WORK-UP** Recommended now Primary care • Within 2–4 weeks

#### Get an in-person assessment

Arrange a primary-care, sports-medicine, or physical-medicine visit within 2–4 weeks. Ask for a focused back and neurologic examination and discuss whether lumbar X-rays or later MRI are appropriate. An uploaded record could not be included, so this plan did not use it.

- **Focused lumbar and neurologic examination** — This helps distinguish mechanical back pain from a nerve, hip, or sacroiliac-region problem.
- **Lumbar imaging if indicated** — Imaging should be chosen based on the examination and symptom course.

- **CONSERVATIVE** Recommended now Primary care • Now and over the next 2–4 weeks

#### Use activity modification while awaiting care

Keep walking and change positions regularly, but temporarily avoid heavy deadlifting, loaded twisting, and gym movements that reproduce the sharp catch. Use comfortable, gradual movement rather than complete rest.

- **Paced activity** — Continue tolerable daily activities and increase them gradually.
- **Workstation and lifting modifications** — Avoid prolonged fixed positions and reduce loads until movement is more comfortable.

## YOUR CARE PATHWAY CONTINUED

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- INTERVENTIONAL** If needed Physiatry • After about 6–12 weeks of guided rehabilitation if still significantly limited

### Consider targeted treatment only if rehabilitation stalls

If a clinician identifies a specific pain generator and symptoms remain function-limiting after a structured rehabilitation trial, a pain or physical-medicine specialist may discuss a targeted procedure.

- Targeted injection** — Potentially temporary relief; only appropriate when the examination supports a specific target.

**When:** Persistent function-limiting pain despite appropriate rehabilitation and a clinically supported target

- SURGICAL** Discouraged

### Do not pursue surgery at this stage

Surgery is not advised for the current localized axial pain pattern without imaging-confirmed pathology that matches the symptoms or a progressive neurologic deficit. Reconsider only if those findings emerge.

**When:** A future examination or imaging shows a clearly concordant structural lesion with progressive neurologic deficit or persistent disabling symptoms

## ACTIVITY GUIDANCE

### Generally okay

- Walking at a comfortable pace
- Changing positions regularly
- Gentle stretching that does not cause sharp or escalating pain
- Normal daily activities within tolerance

### Limit for now

- Heavy deadlifting and loaded spinal flexion
- Twisting while lifting
- Gym movements that reproduce the sharp catch
- Complete bed rest

### Smart modifications

- Use lighter loads and stop before sharp pain during a gradual return to exercise
- Take brief movement breaks during sitting and computer work
- Keep the spine comfortable and avoid combining heavy loading with twisting

## CARING FOR YOURSELF

### Reduce strain

- Return to gym activity gradually rather than restarting heavy deadlifts abruptly.
- Avoid lifting and twisting together while symptoms are active.
- Change position regularly during work and avoid staying in one posture for long periods.
- Use pain and movement tolerance to guide progression rather than complete rest.

### Home care

- Use comfortable heat or ice for short periods if either helps your pain, protecting your skin and following the product instructions.
- Continue gentle walking and easy movement within tolerance.
- Consider acetaminophen or an over-the-counter anti-inflammatory only according to the package label and only if you can take it safely; ask a pharmacist if unsure.
- Use a comfortable sleep position and support your lower back or knees if that reduces strain.

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SpineSense • Clinical Summary • Report FCE3F278

Schema 2.3.0-app • Generated August 2, 2026